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FOCUS GASTROENTEROLOGY

15 April 2026

Dr Alina Yaremenko
Watsonia Central Medical Clinic

Dear Dr Yaremenko,

Re: Ms Alana Trevisan
DOB: 1984-09-04
Address: 422 Plenty Road, PRESTON VIC 3072

Summary

- **Significant improvement in gastrointestinal symptoms following gut-directed hypnotherapy and amitriptyline.**
- **Gradual reintroduction of gluten and fructans starting with sourdough.**
- **Continuation of amitriptyline 25 mg for a twelve-month duration.**
- **No action required by you.**

I had the pleasure of conducting a review with Alana Trevisan via telehealth. Alana reports a marked improvement in her clinical presentation over the last month. She has nearly completed a gut-directed hypnotherapy programme and has been consistently taking amitriptyline at a dose of 25 mg.

Her symptoms of gastroesophageal reflux have diminished significantly; she has not required antacids such as Gaviscon for several months. Furthermore, her bowel habits have stabilised to approximately once daily. She notes a significant reduction in the alternation between constipation and diarrhoea, and she no longer experiences significant abdominal pain. While occasional bloating and flatulence occur, these episodes are infrequent and resolve quickly.

Regarding her diet, Alana has begun to reintroduce dairy with improved tolerance. She has remained on a restricted gluten-free diet but is now prepared to begin a reintroduction phase. We discussed the distinction between gluten and fructans as potential triggers. I have advised her to begin this process using sourdough bread, given its lower fructan content. She will follow a gradual dose-escalation strategy, maintaining the same portion for three to four days to monitor for any

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St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

cumulative symptomatic effects.

Impression and Plan

Alana's positive response is likely a result of the synergistic effects of her neuromodulation and the hypnotherapy. We have agreed to continue the amitriptyline 25 mg for a total duration of twelve months. If she experiences a resurgence of symptoms during dietary reintroduction, I have advised that she may increase her dose to 37.5 mg (one and a half tablets), noting the therapeutic lag of several weeks.

Alana will proceed with dietary challenges independently but may re-consult her dietitian, Erin, should the reintroduction prove difficult. I have provided her with a repeat prescription for amitriptyline and will review her again in six months' time.

Kind regards

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Chamara Basnayake